

Q I am lactose-intolerant. What can I do about my calcium intake?

Your ability to digest lactose may actually improve while you are pregnant, or you may need to take a calcium supplement to make sure you get enough calcium.

Calcium is crucial: it helps to build your baby's bones and teeth and prevents your bones from weakening. Interestingly, studies show that lactose intolerance eases off in around 30 to 50 percent of pregnancies. So, to begin with, consult your doctor about how to reintroduce a tiny amount of cow's milk into your diet and measure its effects, and follow his or her guidance before you begin supplementation. If you remain intolerant, you may need to take a calcium supplement, as well as vitamin B₁₂ and D supplements, which are also found in dairy products. In the meantime, increase your intake of other calcium-rich foods.

CALCIUM-RICH FOODS TO EAT IF YOU ARE LACTOSE-INTOLERANT

Include a few of these calcium-rich foods in your diet each day; the daily recommendation is 1,000 milligrams of calcium.

FOOD	SERVING AMOUNT	CALCIUM CONTENT
Canned sardines	2 oz (60 g)	240 mg
Water cress	4 oz (120 g) raw	188 mg
Wakame	3½ oz (100 g) raw	150 mg
Green beans	7 oz (200 g) cooked	132 mg
Tofu	4 oz (120 g) raw	126 mg
Broccoli	4 oz (120 g) raw	112 mg
Rhubarb	4 oz (120 g) raw	103 mg
Chickpeas	7 oz (200 g) cooked	99 mg



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Q I'm Muslim and I plan to observe Ramadan. Will fasting harm my baby?

Any kind of fast is not recommended during pregnancy—your body and your baby need a constant supply of nutrition and water in order to remain healthy and develop properly. Studies show that fasting can lead to premature birth, low birth weight, and neurological problems in babies. However, many Muslim women choose to observe the Ramadan fast. First, consult your doctor to discuss your individual situation and your baby's health, and then fast only if you have a healthy weight, good energy levels, no known conditions, and you feel strong. Islamic law does permit pregnant women not to fast; you can make up the days after the birth or compensate by giving to charity. So, if at any time you feel dizzy, dehydrated, or otherwise unwell, or if you think your baby has stopped moving, stop the fast immediately and consult your doctor.

Q Is there anything special I need to worry about as a pescetarian?

This semivegetarian diet, which excludes meat and poultry and may include dairy products and eggs in addition to fish and shellfish, shouldn't exclude any natural nutrients from your diet. Fish is an ideal form of protein; it is rich in B-complex vitamins, calcium, potassium, iron, zinc, phosphorous, and selenium, and the omega-3 fats that oily fish such as halibut, salmon, tuna, mackerel, and sardines contain are crucial for heart and brain health. However, the current medical advice is that you should eat only two or three servings of fish per week (8–12 ounces), choosing fish varieties that are lower in mercury, like salmon, light canned tuna and tilapia. (Too much mercury can be harmful to your unborn baby's development. Avoid swordfish, tilefish, shark, and mackerel, which are high in mercury.) Limit white albacore tuna to 6 ounces per week. Avoid raw shellfish, too.

Q I've been following a low-carb diet. Is it safe to continue this diet?

Without good medical reason (such as an allergy, intolerance, or other food-related autoimmune condition), there is no justification for restricting or eliminating any major nutrient during pregnancy. Carbohydrates are an important source of energy, fiber, iron, and B-vitamins, which makes them essential for your health and that of your baby. If you're worried about weight gain, remember that, gram for gram, carbohydrates have fewer calories (1g of carbohydrate gives 4 calories of energy) than fat-rich foods (9 calories per gram). Carbohydrates also keep you feeling fuller longer, so they will actually help prevent any tendency you may have to snack.

Q I'm overweight. Would it make sense to eat fewer calories now that I'm pregnant?

Usually, dieting to lose weight while pregnant is not advised because it can deprive your baby of essential nutrients, but if your BMI was over 30 to begin with (see p.51), it can be a good idea to manage your weight. If you were already obese, there are specific risks to pregnancy—such as gestational diabetes, preeclampsia, having a large baby, stillbirth, and increased likelihood of cesarean section. Work with your doctor to create a program to maintain your weight throughout your pregnancy. Too much calorie restriction is not ideal—especially if you were underweight before becoming pregnant—since this increases the risk of having a small, or low birth weight baby, which is linked to developmental issues.

Carbohydrates are essential: if your body doesn't get enough fuel from carbs, it begins to metabolize your fat reserves and release toxins called PCBs that may harm your baby's development.